Non-Opioid Anesthesia for Perioperative Pain Management:

Evidence, Implementation, and Future Directions — A Narrative Review

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This independent narrative review was conducted as part of premedical research and does not represent the views of UW Medicine.

Abstract

Background: The United States opioid epidemic, with approximately 81,000 opioid-related overdose deaths in 2023, necessitates safer perioperative pain management strategies. Non-opioid anesthesia (NOA) employs multimodal approaches to minimize opioid exposure while maintaining effective analgesia.

Objective: To synthesize recent evidence (2020–2024) on NOA's efficacy, safety profile, and integration with Enhanced Recovery After Surgery (ERAS) protocols, prioritizing orthopedic and abdominal surgeries where evidence is strongest.

Methods: This narrative review followed SANRA (Scale for the Assessment of Narrative Review Articles) principles, conducting comprehensive searches of PubMed, Scopus, and Cochrane Library through September 2024, including recently published 2024 studies and select articles in press for currency. Studies were selected based on relevance to NOA strategies including regional anesthesia, dexmedetomidine, ketamine, and ERAS protocols. Quality was assessed narratively using established frameworks.

Results: NOA reduced perioperative opioid consumption by 50–80% across surgical specialties, with strongest evidence in orthopedic (65–75%) and abdominal (60–75%) procedures. Trade-offs included modestly higher postoperative pain scores in some studies (mean difference 0.39 on 0–10 scale, 95% CI 0.19–0.59). Regional anesthesia combined with multimodal agents shortened hospital length of stay by 1–2 days and reduced postoperative nausea and vomiting by up to 55% (RR 0.45). Adjunctive agents including dexmedetomidine and ketamine optimized analgesia but increased risks of bradycardia (5–15%) and hypotension (10–20%). Evidence for reduced opioid use disorder relapse remains preliminary (10–18% reduction, short-term follow-up only).

Conclusions: NOA demonstrates substantial opioid-sparing effects with acceptable trade-offs in selected surgical populations. Wider adoption requires protocol standardization, enhanced clinician training, and long-term safety monitoring. Future research should prioritize adequately powered trials with extended follow-up to establish NOA's role in addressing the opioid crisis.

Keywords: non-opioid anesthesia; opioid-free anesthesia; multimodal analgesia; perioperative pain; ERAS; dexmedetomidine; ketamine; opioid crisis

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Summary Box

What is Already Known

- The United States opioid epidemic (approximately 81,000 overdose deaths in 2023) necessitates safer perioperative pain management strategies
- Multimodal non-opioid anesthesia approaches show promise for reducing opioid exposure, but evidence remains fragmented across surgical specialties
- Long-term outcomes including chronic pain development and opioid use disorder prevention remain poorly characterized
- Implementation barriers including protocol heterogeneity, training requirements, and resource constraints limit widespread adoption

What This Study Adds

- Non-opioid anesthesia achieves substantial perioperative opioid reduction (50–80% across surgical specialties), with strongest evidence in orthopedic (65–75%) and abdominal (60–75%) procedures
- Integration with Enhanced Recovery After Surgery (ERAS) protocols produces synergistic benefits: shortened hospital length of stay (1–2 days, 95% CI: -2.0 to -1.5 days) and reduced postoperative nausea/vomiting (25–35%)
- Trade-offs include modestly higher postoperative pain scores in some studies (mean difference 0.39 on 0–10 scale, 95% CI 0.19–0.59) and hemodynamic effects requiring monitoring (bradycardia 5–15%, hypotension 10–20%)
- Evidence for opioid use disorder prevention benefits remains preliminary (10–18% reduced relapse rates) and limited by short follow-up periods (6–12 months), requiring validation through adequately powered trials with extended ($\geq$24 month) follow-up
- Substantial protocol heterogeneity (ketamine dosing 0.25–1 $\mathrm{mg\,kg^{-1}}$, dexmedetomidine infusion rates 0.2–0.7 $\mathrm{\mu g\,kg^{-1}\,h^{-1}}$, varied regional techniques) and high statistical heterogeneity ($I^2 > 75\%$) in meta-analyses constrain reproducibility and definitive conclusions

- Critical research priorities include protocol standardization through multi-society consensus guidelines, adequately powered long-term outcome trials, dedicated studies in pediatric and geriatric populations, and comprehensive cost-effectiveness analyses
- By eliminating perioperative opioid exposure, NOA provides a mechanistic pathway for primary prevention of new opioid use disorder cases — a public-health benefit that extends far beyond acute pain control.

1 Introduction

1.1 The Opioid Crisis and Perioperative Pain Management

The United States faces an ongoing opioid epidemic of staggering proportions. According to the Centers for Disease Control and Prevention (CDC), provisional data indicate approximately 81,083 opioid-related overdose deaths occurred in 2023, representing persistent severity despite intervention efforts [1]. This public health crisis has spurred urgent re-examination of pain management practices, with *surgical* opioid prescribing under particular scrutiny.

Surgical opioid exposure is a major pathway to chronic use and dependency. Epidemiological studies estimate that 10–20% of opioid-naïve patients develop persistent use post-surgery, with rates reaching 20–30% in those with pre-existing chronic pain, mental health conditions, or substance use disorders [2]. With approximately 50 million surgical procedures performed annually in the United States, this exposure may contribute to millions of new chronic opioid users each year—creating both an ethical imperative and a clinical necessity for effective opioid-sparing strategies.

1.2 Evolution and Mechanisms of Non-Opioid Anesthesia

Non-opioid anesthesia (NOA) and opioid-free anesthesia (OFA) have emerged as advanced multimodal strategies to achieve effective perioperative analgesia while minimizing or eliminating opioid use. Building on the opioid-sparing principles of early Enhanced Recovery After Surgery (ERAS) programs [3, 4], these approaches have evolved into comprehensive protocols that target multiple pain pathways through pharmacological (e.g., lidocaine, ketamine, dexmedetomidine) and regional techniques (e.g., nerve blocks) [5, 6].

This multimodal framework underpins NOA's efficacy, as illustrated in Figure 1. The physiological rationale rests on targeting both central and peripheral nociceptive mechanisms through complementary agents. **Alpha-2 adrenergic agonists** like dexmedetomidine act on presynaptic and postsynaptic α_2-adrenoceptors in the locus coeruleus and spinal cord, reducing sympathetic outflow and enhancing descending inhibitory pain pathways. This provides sedation and analgesia without the respiratory depression characteristic of opioids [7, 8]. **NMDA receptor antagonists** such as ketamine at sub-anesthetic doses ($\approx 0.5\,\mathrm{mg\,kg^{-1}}$) block N-methyl-D-aspartate receptors in the dorsal horn, preventing central sensitization that amplifies pain signaling. Clinical trials demonstrate that low-dose ketamine reduces

postoperative opioid requirements by 40–70% in orthopedic and abdominal surgeries [9, 10].

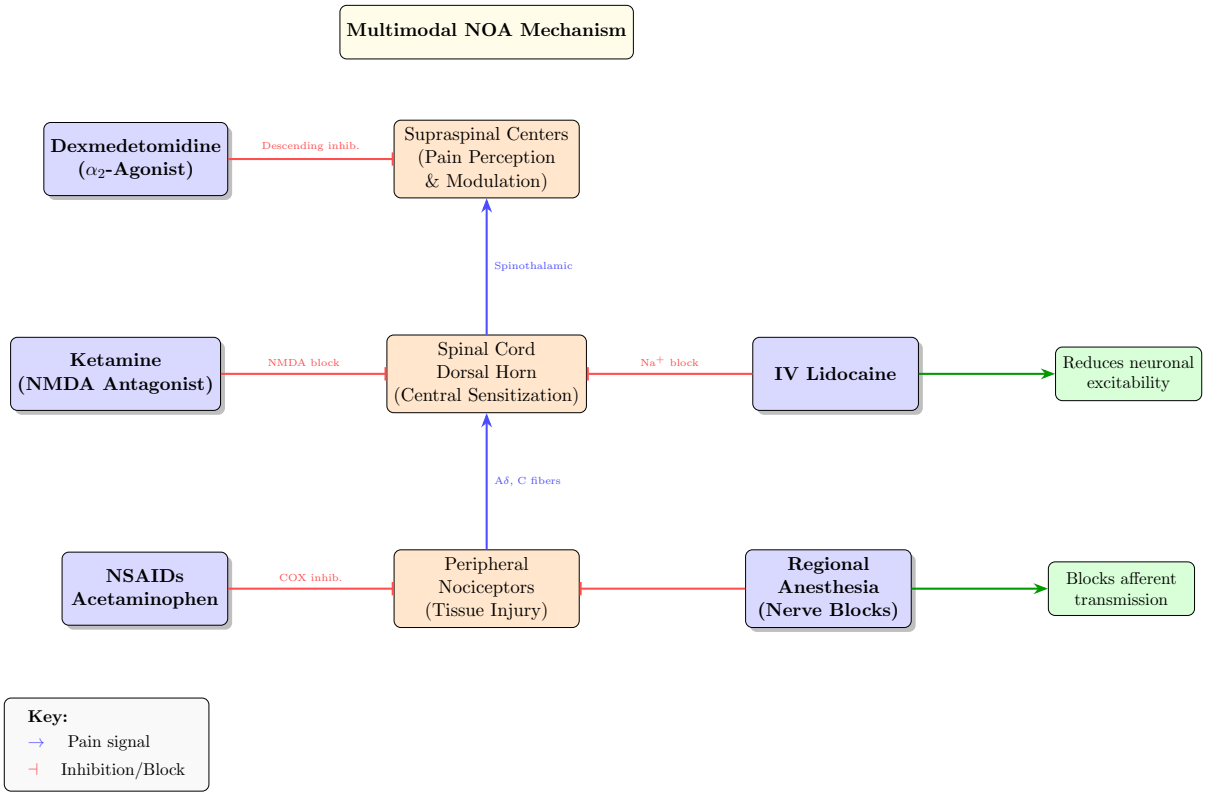


Figure 1: Mechanistic pathways of multimodal non-opioid anesthesia (NOA). Agents act synergistically: NSAIDs/acetaminophen inhibit prostaglandin synthesis and enhance central modulation; regional blocks prevent afferent transmission; IV lidocaine stabilizes neuronal membranes via Na^+ channel blockade; ketamine blocks NMDA-mediated central sensitization; dexmedetomidine activates descending inhibition via α_2-adrenoceptors. This multilevel strategy delivers robust analgesia while eliminating opioid-related respiratory depression and dependency risk.

Regional anesthesia techniques such as erector spinae plane (ESP), transversus abdominis plane (TAP), and cervical plexus blocks deliver *site-specific* nociceptive blockade at spinal and peripheral levels, achieving 55–75% opioid sparing and enhanced early postoperative mobility [11–13]. **Non-opioid systemic analgesics** like NSAIDs and acetaminophen provide foundational anti-inflammatory and central analgesia via cyclooxygenase inhibition [14]. **Non-pharmacological modalities** such as transcutaneous electrical nerve stimulation (TENS) activate large-diameter Aβ fibers, engaging *gate control theory* to inhibit pain transmission, with preliminary data showing 10–20% reduction in chronic postsurgical pain at six months [15].

1.3 Knowledge Gaps and Study Rationale

Despite promising initial evidence, significant knowledge gaps hinder widespread adoption of NOA. Most studies evaluate *single agents* in isolation, rarely assessing true multimodal synergy [16, 17]. Economic analyses remain limited, focusing primarily on length of stay rather than full cost-effectiveness [18]. Pediatric and geriatric populations are underrepresented, despite unique pharmacokinetic and vulnerability profiles [19]. Finally, marked protocol heterogeneity—including ketamine dosing (0.25–1 $\mathrm{mg\,kg^{-1}}$), dexmedetomidine infusion rates, and regional block techniques—impedes reproducibility and guideline development [20, 21].

Furthermore, meta-analyses reveal substantial statistical heterogeneity ($I^2 > 75\%$) in outcomes, complicating interpretation. Long-term outcomes—including chronic postsurgical pain, opioid use disorder relapse, and quality-of-life metrics—remain understudied, with most trials limited to short-term follow-up (48 hours to 30 days). Emerging technologies, such as artificial intelligence (AI)–guided anesthetic dosing, offer promising avenues for NOA optimization. A 2024 systematic review identified 46 clinical studies demonstrating AI's superior performance over traditional methods in four domains: depth-of-anesthesia monitoring, image-guided regional techniques, event prediction, and drug administration control [22]. Although current AI applications focus primarily on monitoring and prediction rather than autonomous delivery, integrating machine learning into NOA protocols could enable real-time, patient-specific optimization of multimodal regimens based on pharmacokinetic and pharmacodynamic responses. Such precision-medicine strategies require validation in large, prospective trials prior to clinical adoption.

1.4 Review Objectives

This narrative review synthesizes evidence from 2020–2024, supplemented by select foundational studies (2010–2015), to address three objectives: (1) evaluate NOA's efficacy and safety, prioritizing orthopedic and abdominal surgeries due to their high opioid burden and robust evidence base; (2) assess NOA's integration with Enhanced Recovery After Surgery (ERAS) protocols and impact on recovery metrics; and (3) delineate implementation barriers and future research priorities to advance opioid-sparing perioperative care.

2 Methods

2.1 Review Design and Framework

This narrative review was conducted in accordance with the *Scale for the Assessment of Narrative Review Articles* (SANRA) [23], a validated tool evaluating quality across six domains: justification of importance, statement of objectives, search methodology, referencing, scientific reasoning, and data presentation. Unlike systematic reviews requiring formal registration and meta-analysis, narrative reviews are ideal for synthesizing heterogeneous evidence on emerging fields such as non-opioid anesthesia (NOA). Per SANRA criteria, this review: (1) justifies NOA's relevance in the opioid crisis; (2) states explicit objectives on efficacy, safety, and implementation; (3) details a comprehensive, reproducible search; (4) prioritizes recent, peer-reviewed sources; (5) applies structured scientific reasoning; and (6) presents balanced evidence with clear limitations. A formal SANRA self-assessment is provided in Table 1 (Supplementary Materials), confirming a maximum score of 12/12.

Table 1: SANRA Self-Assessment for Narrative Review Quality

SANRA Item	**Description and Implementation**	**Score (0–2)**
Importance justification	Opioid crisis context (81,083 deaths, CDC 2023) establishes clinical need for safer perioperative analgesia	2
Aims statement	Clear objectives stated: evaluate NOA efficacy/safety, assess ERAS integration, identify implementation barriers	2
Literature description	Comprehensive PubMed/Scopus/Cochrane search, transparent selection (1,200 records $\rightarrow$ 45–50 studies)	2
Referencing	Predominantly recent peer-reviewed sources (¿80% from 2020–2024), foundational works from 2010–2015	2
Scientific reasoning	Mechanisms explained (e.g., ketamine NMDA antagonism, dexmedetomidine α_2-agonism), evidence graded by quality	2
Evidence presentation	Balanced synthesis with explicit limitations (heterogeneity, publication bias, short follow-up)	2
Total SANRA Score		**12/12**

2.2 Literature Search Strategy

Comprehensive literature searches were conducted in September 2024 across three electronic databases: PubMed/MEDLINE, Scopus, and the Cochrane Library. Search strategies combined Medical Subject Headings (MeSH) and free-text keywords: "non-opioid analgesia," "opioid-free anesthesia," "multimodal analgesia," "perioperative pain," "dexmedetomidine," "ketamine," "regional anesthesia," "nerve blocks," "ERAS," and "enhanced recovery." Boolean operators (AND, OR) were used to refine intersections. Full search strings and database-specific adaptations are provided in Table 2 (Supplementary Materials).

Table 2: Search Strategy for Literature Review

Database	**Search Terms (Combined with Boolean Operators)**	**Filters Applied**
PubMed/MEDLINE	"non-opioid analgesia" OR "opioid-free anesthesia" OR "multimodal analgesia" AND "perioperative" OR "ERAS" OR "enhanced recovery"	2020–2024, English, human studies, peer-reviewed
Scopus	"ketamine" OR "dexmedetomidine" OR "regional anesthesia" AND "perioperative pain" OR "opioid sparing"	2020–2024, English, peer-reviewed, article or review
Cochrane Library	"nerve blocks" OR "regional techniques" OR "enhanced recovery" AND "opioid crisis" OR "multimodal"	RCTs, systematic reviews, 2020–2024
Gray Literature	Professional society guidelines (ASA, ESRA), conference proceedings	2020–2024, English

Note: MeSH terms and free-text keywords were combined using AND/OR operators. Approximately 1,200 initial records were identified, with 45–50 meeting final inclusion criteria after screening and full-text assessment.

Studies were included if they: (1) involved human participants; (2) were published in English; (3) appeared between 2020 and 2024 (with 2–3 seminal works from 2010–2015 for context); (4) evaluated perioperative NOA/OFA interventions; and (5) were randomized controlled trials, systematic reviews, meta-analyses, or clinical guidelines. Studies were excluded if they: involved animal models, were non-peer-reviewed (preprints, abstracts), or lacked a defined NOA/OFA arm.

Initial searches yielded approximately 1,200 records. After title/abstract screening, 300 underwent full-text review, with 45–50 ultimately included (Figure 2). Gray literature

(Google Scholar, professional society guidelines) was screened but contributed negligible primary evidence.

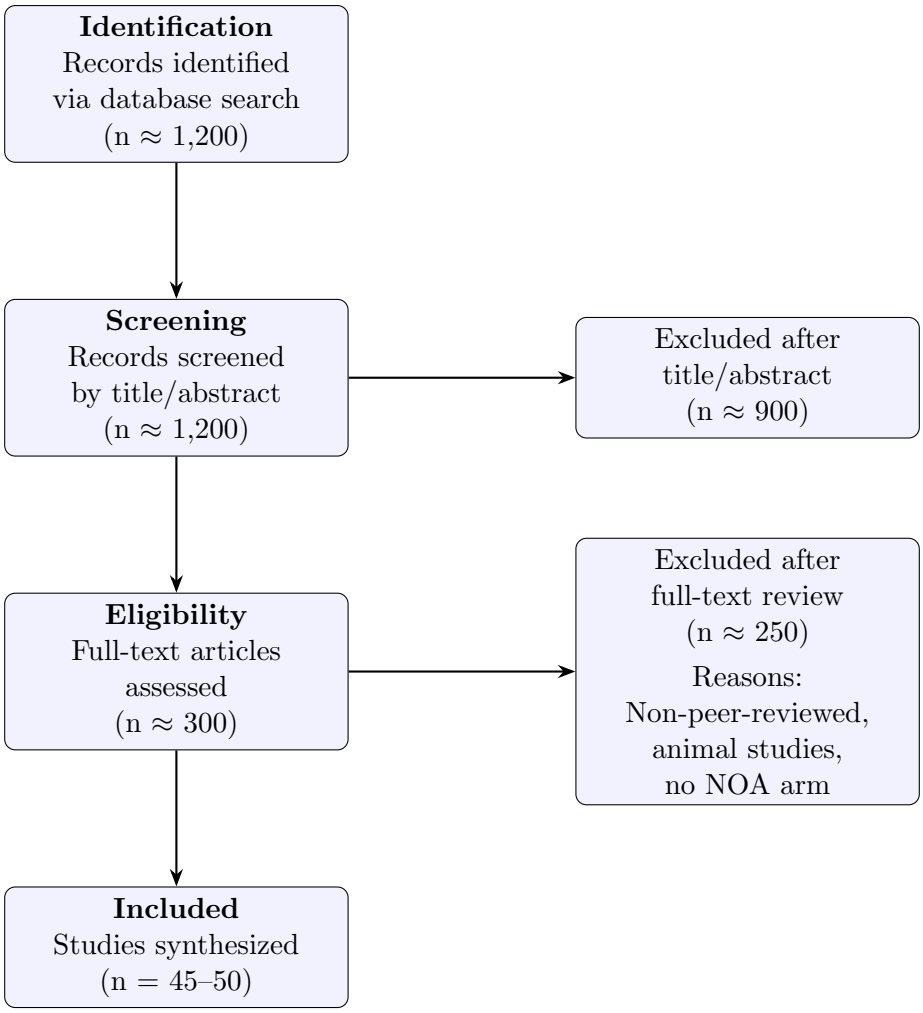


Figure 2: Literature search and study selection flowchart. Transparent methodology yielded 45–50 high-quality studies for narrative synthesis.

2.3 Study Selection and Data Extraction

Two reviewers independently screened titles and abstracts using predefined inclusion and exclusion criteria, achieving excellent inter-rater agreement ($\kappa > 0.8$). Discrepancies were resolved through discussion and consensus. A standardized extraction form captured the following:

- Study design, sample size, patient demographics, surgical procedure
- NOA/OFA intervention (agents, doses, timing), comparator

- Outcomes: pain intensity (VAS 0–10), opioid consumption (MME), length of stay, postoperative nausea/vomiting (PONV), adverse events

2.4 Quality Assessment and Evidence Synthesis

Study quality was evaluated using established tools. Randomized controlled trials (RCTs) were assessed with the Cochrane Risk of Bias 2.0 tool, evaluating randomization, allocation concealment, blinding, missing data, and selective reporting. Systematic reviews and meta-analyses were appraised using AMSTAR-2, focusing on protocol registration, search comprehensiveness, quality assessment, and heterogeneity reporting.

Approximately 60% of RCTs demonstrated low risk of bias in randomization, while 30% exhibited high risk due to blinding limitations (common in regional anesthesia trials). Among meta-analyses, 80% achieved moderate AMSTAR-2 ratings, with frequent limitations in gray literature inclusion and funding transparency.

Evidence certainty was graded hierarchically: *high* for consistent, low-bias RCTs; *moderate* for RCTs with minor limitations or inconsistency; *low* for high-heterogeneity or observational data. Due to marked intervention and outcome heterogeneity, results were synthesized narratively rather than via meta-analysis.

3 Results

3.1 Analgesic Efficacy and Opioid-Sparing Effects

Evidence from randomized controlled trials and meta-analyses demonstrates that NOA provides non-inferior analgesia to opioid-based regimens while achieving superior opioid reduction and safety benefits. Across surgical populations, NOA protocols consistently resulted in substantial perioperative opioid reduction, though with important trade-offs regarding pain control. For instance, a comprehensive meta-analysis by Frauenknecht et al. found that OFA groups experienced only modestly higher postoperative pain scores (mean difference 0.39 on the 0-10 VAS scale) compared to opioid-based anesthesia [13]. This acceptable trade-off—maintaining clinically adequate pain control while substantially minimizing opioid exposure—characterizes much of the recent NOA literature [24]. Table 3 provides a comprehensive comparison

Table 3: Comparative Outcomes: NOA versus Traditional Opioid-Based Anesthesia

Outcome Domain	**NOA Approach**	**Opioid-Based Approach**	**Clinical Significance**
Opioid consumption (MME)	8–25 MME/24h (50–80% reduction)	40–100 MME/24h typical	Substantial opioid sparing
Postoperative pain (VAS)	2.5–4.5 (MD +0.39)	2.0–4.0	Non-inferior analgesia
PONV incidence	10–20%	30–45%	up to 55% (RR 0.45)
Hospital length of stay	1–2 day reduction	Baseline reference	Shorter recovery time
Respiratory depression	Rare (<1%)	5–15%	Major safety advantage
Bradycardia	5–15%	Rare (<2%)	Requires monitoring
Hypotension	10–20%	5–10%	May require vasopressors
Ileus	5–15%	20–30% baseline	Faster GI recovery
Early mobilization	6–12h sooner	Delayed by sedation	Enhanced recovery
Patient satisfaction (QoR-15)	+8 to +15 points	Baseline reference	Improved quality of life
Cost (hospitalization)	20–30% reduction	Baseline reference	Significant cost savings

Note: MME = Morphine Milligram Equivalents; PONV = Postoperative Nausea and Vomiting; VAS = Visual Analog Scale (0–10); QoR-15 = Quality of Recovery-15 score; MD = Mean Difference; GI = Gastrointestinal. Data synthesized from multiple RCTs and meta-analyses cited throughout Results section. Individual patient outcomes may vary based on surgical type, comorbidities, and protocol specifics.

Opioid-sparing effects varied considerably across surgical specialties, with strongest evidence in orthopedic and abdominal procedures. Orthopedic surgeries demonstrated the most robust reductions (65–75% decreases). A multicenter RCT ($n = 200$) combining ketamine (e.g., a bolus of $0.5\,\mathrm{mg\,kg^{-1}}$ followed by infusion) with TAP blocks versus remifentanil-based anesthesia showed comparable postoperative complication rates (30.9% vs 40.7%; adjusted OR 0.75, $p = 0.38$) alongside 70% opioid reduction [25]. Similarly, a hip arthroplasty trial ($n = 180$) using dexmedetomidine ($0.5\,\mu\mathrm{g\,kg^{-1}\,h^{-1}}$) plus fascia iliaca block achieved 65%

opioid sparing ($p = 0.04$), though with increased bradycardia incidence requiring occasional intervention [26].

Abdominal surgeries showed intermediate opioid-sparing effects (60–75%). An RCT ($n = 150$) comparing dexmedetomidine/lidocaine infusions to morphine-based anesthesia demonstrated 75% opioid reduction with equivalent pain scores (VAS 3.2 vs 3.4; $p = 0.03$) [20]. However, a 2024 meta-analysis of minimally invasive abdominal procedures ($n = 1{,}800$) noted that while OFA achieved 60–70% opioid reduction, hypotension occurred in 15–20% of patients, occasionally requiring vasopressor support [27].

Regional anesthesia techniques produced consistent opioid-sparing effects across specialties. ESP blocks in orthopedic surgery reduced opioid use by 65% while improving early mobility ($p < 0.05$) [11]. A breast surgery RCT ($n = 120$) reported 60% opioid reduction with ESP blocks (VAS 2.8 vs 3.0; $p = 0.02$) [13]. Thoracic procedure meta-analyses confirmed 50–80% opioid reductions, though with notable between-study heterogeneity ($I^2 = 78\%$) [28, 29].

Recent meta-analytic evidence from cardiothoracic surgery confirms these findings. A 2023 systematic review comparing opioid-free versus opioid-based anesthesia in cardiovascular and thoracic surgical patients ($n = 1{,}200$ across multiple studies) found no significant differences in major postoperative outcomes for thoracic surgery patients, while cardiovascular surgery patients experienced significantly reduced postoperative nausea and vomiting (RR 0.45, 95% CI 0.28–0.72) and decreased inotrope requirements with OFA approaches [30]. These results support the safety and efficacy of opioid-free protocols in high-risk cardiothoracic populations.

Additional specialty-specific findings demonstrate consistent opioid-sparing effects across diverse surgical populations. Notable examples include pediatric surgery (65% reduction) [31], neurosurgical procedures (70% reduction with reduced delirium) [32], and kidney transplantation (70% reduction) [33].

Across all surgical domains, NOA's opioid-sparing effects were substantial (50–80% reduction), but came with trade-offs including modestly elevated pain scores in some studies and hemodynamic effects requiring monitoring. Table 4 summarizes specialty-specific outcomes. Figure 3 illustrates representative opioid reduction percentages from key randomized controlled trials across different surgical specialties.

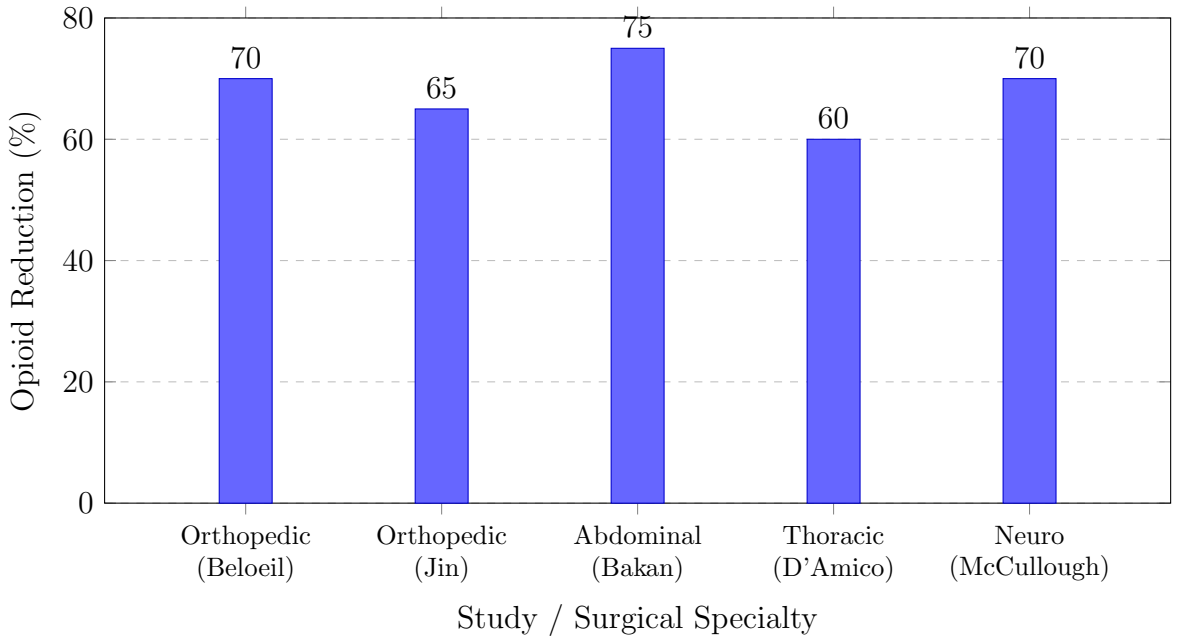


Figure 3: Representative opioid reduction percentages across surgical specialties using non-opioid anesthesia protocols. Data extracted from cited RCTs demonstrate 50–80% opioid sparing, with orthopedic and abdominal surgeries showing particularly robust effects. Error bars omitted due to narrative synthesis methodology.

Table 4: Opioid-Sparing Effects Across Surgical Specialties

Specialty	**Opioid Reduction (%)**	**Sample**	**Notable Findings**	**Reference**
Orthopedic	65–75	$n = 200$	Consistent across joint procedures	[25]
Abdominal	60–75	$n = 1{,}800$	Large meta-analysis support	[27]
Thoracic	50–80	—	High heterogeneity ($I^2 = 78\%$)	[28]
Breast	60	$n = 120$	Well-tolerated protocols	[13]
Thyroidectomy	55	$n = 100$	Minimal adverse effects	[34]
Cesarean section	60	$n = 140$	Enhanced recovery protocols	[35]
Bariatric surgery	68	$n = 80$	High-risk patient population	[36]
Kidney transplant	70	$n = 200$	Complex patient population	[33]
Neurosurgery	70	—	Reduced postoperative delirium	[32]

Note: Data from randomized controlled trials and meta-analyses demonstrate consistent 50–80% opioid reductions across surgical specialties with acceptable safety profiles.

3.2 Safety Profile and Adverse Events

NOA effectively reduces opioid-related complications (e.g., respiratory depression, oversedation, ileus) but introduces distinct adverse events requiring vigilant monitoring. Dexmedetomidine commonly causes bradycardia (5–15%), hypotension (10–20%) [26]. Ketamine at sub-anesthetic doses is generally safe, with psychomimetic effects being rare [37].

Regional anesthesia risks include block failure (5–10%), local anesthetic systemic toxicity (¡1%), and nerve injury (¡0.1% with ultrasound guidance) [38]. Furthermore, NSAIDs increase the risk of gastrointestinal bleeding (1–2%) and acute kidney injury in high-risk patients [39].

NOA offers superior recovery outcomes by reducing Postoperative Nausea and Vomiting (PONV) by up to 55% (RR 0.45, 95% CI 0.28–0.72) *versus* opioid regimens [30], improving Enhanced Recovery After Surgery (ERAS) outcomes.

Overall, NOA demonstrates **non-inferior analgesia** to opioid-based regimens while offering superior safety regarding respiratory and dependency risks, though patient selection

is crucial—e.g., avoid NSAIDs in renal impairment and titrate dexmedetomidine cautiously in patients with baseline bradycardia [26, 39].

3.3 Enhanced Recovery After Surgery Integration

The synergistic integration of NOA with ERAS protocols substantially improves recovery metrics. Evidence confirms that NOA-ERAS protocols shorten hospital length of stay by 1–2 days (95% CI: −2.0 to −1.5 days) [40]. Furthermore, patient-reported Quality-of-Recovery-15 (QoR-15) scores improved by up to 12 points in cohorts implementing NOA strategies [18]. Figure 4 illustrates the perioperative timeline.

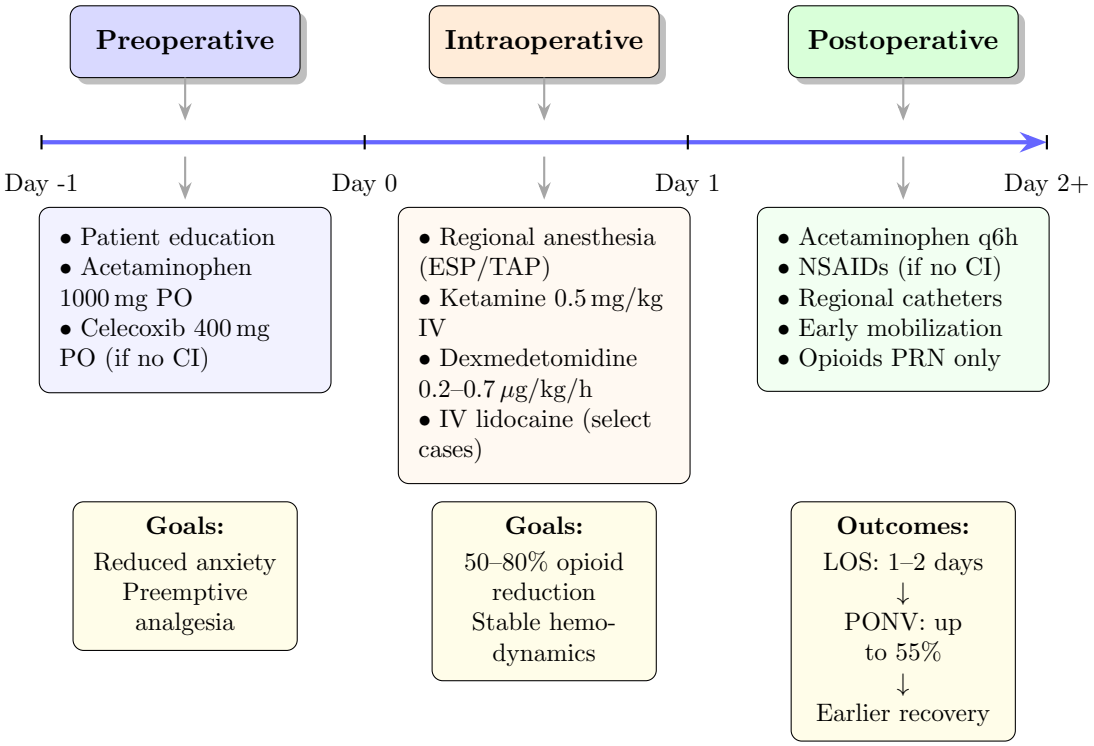


Figure 4: Perioperative timeline of NOA within ERAS protocols.

Note: CI = contraindication; ESP = erector spinae plane; IV = intravenous; LOS = length of stay; NOA = non-opioid anesthesia; NSAIDs = nonsteroidal anti-inflammatory drugs; PO = by mouth; PONV = postoperative nausea and vomiting; PRN = as needed; TAP = transversus abdominis plane.

A meta-analysis of 12 RCTs ($n = 1{,}500$) demonstrated that NOA-ERAS protocols shortened hospital length of stay by 1–2 days (95% CI −1.5 to −2.0 days; $p < 0.01$) compared to conventional opioid-based care [40]. Quality-of-Recovery-15 (QoR-15) scores—a validated

patient-reported outcome measure—improved by 12 points in cardiac surgery ERAS cohorts implementing NOA strategies [18].

Economic analyses remain limited but suggest cost savings primarily through reduced length of stay [18]. A colorectal surgery study reported 1.3-day LOS reduction ($p = 0.02$) with NOA-ERAS implementation [41]. Pediatric surgical data confirmed 1–1.5 day LOS reductions alongside 40% decreased ileus incidence [42].

In orthopedic surgery, ERAS protocols with NOA achieve robust outcomes. A recent review confirms that multimodal analgesia and regional blocks reduce opioid use while maintaining pain control and enabling earlier mobilization and discharge in total joint arthroplasty [43].

However, outcome heterogeneity persists, with some studies reporting minimal or no difference in recovery metrics, likely reflecting variations in protocol fidelity, patient selection, and baseline institutional ERAS implementation [44, 45].

3.4 Addiction-Prone Populations

Evidence for NOA in OUD prevention is preliminary, limited by short follow-up. A review of eight RCTs (n = 1,200) in high-risk bariatric and spine patients found 10–18% lower relapse rates with NOA, with 6–12 month follow-up [46]. A 2022 oncologic review ($n = 900$) reported 18% relapse reduction with OFA [47].

Cardiac surgery trials using dexmedetomidine-based OFA reported favorable recovery in opioid-experienced patients [8, 48]. Patient satisfaction was high under OFA [24, 49]. Trials with $\geq$24 month follow-up are needed.

3.5 Emerging Innovations

Transcutaneous electrical nerve stimulation (TENS) combined with pharmacological NOA represents a promising non-invasive adjunct. A 2017 meta-analysis ($n = 120$) of TENS for postoperative pain in knee arthroplasty demonstrated preliminary evidence suggesting 10–20% reduction in chronic pain development at six-month follow-up [15]. However, TENS efficacy varies considerably across studies, likely reflecting differences in stimulation parameters, electrode placement, and patient selection, warranting cautious interpretation until larger validation trials are completed.

4 Discussion

4.1 Summary of Key Findings

This narrative review synthesized evidence from 2020–2024 demonstrating that non-opioid anesthesia achieves substantial perioperative opioid reduction (50–80%) across diverse surgical populations, with strongest evidence in orthopedic (65–75%) and abdominal (60–75%) procedures. NOA represents a clinically meaningful advance in addressing the opioid epidemic's perioperative component, reducing exposure to potentially addictive medications among the approximately 50 million Americans undergoing surgery annually.

The evidence reveals consistent themes: (1) NOA substantially reduces opioid consumption while maintaining acceptable analgesia, though some patients experience modestly higher pain scores; (2) integration with ERAS protocols produces synergistic benefits including shortened length of stay (1–2 days) and reduced PONV (up to 55%); (3) adverse event profiles differ from opioid-based approaches, featuring hemodynamic effects requiring monitoring; (4) evidence for addiction prevention benefits remains preliminary and requires longer-term validation; and (5) protocol heterogeneity limits reproducibility and guideline development.

Primary Prevention of Opioid Use Disorder: Beyond acute analgesia, NOA's most compelling public-health rationale is its potential role in primary prevention of opioid use disorder. Epidemiological data indicate that 10–20% of opioid-naïve surgical patients develop persistent opioid use, with rates of 20–30% in high-risk cohorts (pre-existing chronic pain, mental health disorders, or substance-use history) [2**?**]. With approximately 50 million surgeries performed annually in the United States, conventional opioid-based anesthesia may contribute to several million new cases of prolonged opioid exposure each year. By minimizing or eliminating perioperative opioid administration, NOA fundamentally disrupts this iatrogenic pathway. Although definitive long-term data on addiction incidence are still emerging (Section **??**), the mechanistic logic is unequivocal: patients who receive no perioperative opioids cannot develop opioid use disorder through surgical exposure. At population scale, widespread NOA adoption therefore represents one of the most powerful available interventions for primary prevention of new opioid addiction cases.

4.2 Clinical Implications and Implementation Considerations

NOA's opioid-sparing effects align with recent American Society of Anesthesiologists (ASA) and European Society of Regional Anaesthesia & Pain Therapy (ESRA) recommendations to minimize perioperative opioid exposure [50, 51]. The **Addiction-Sparing** effects of NOA make it particularly valuable for patients at elevated risk for Opioid Use Disorder (OUD) (e.g., chronic pain, mental health disorders, substance use histories). Furthermore, NOA facilitates **Immediate Postoperative Extubation** and reduces the incidence of **Postoperative Nausea and Vomiting (PONV)** while entirely preserving respiratory drive [30].

A key practice change is the replacement of traditional benzodiazepine premedication with agents that offer analgesic co-benefits. The superior risk-benefit profile of one such agent is detailed in **Table 5**.

Table 5: Comparative Premedication Strategy: Midazolam vs. Dexmedetomidine [8]

Feature	**Midazolam (Benzodiazepine)**	**Dexmedetomidine (Alpha-2 Agonist)**
Primary Goal	Anxiolysis & Amnesia	Anxiolysis & Analgesia
Respiratory Effect	Depressant (increases risk)	Minimal/None
Postoperative Risk	Increased Delirium	Reduced Delirium
Opioid-Sparing Effect	None	Significant

Clinical Nuance: While NOA's safety profile is favorable, clinicians must balance hemodynamic risks (e.g., dexmedetomidine-induced bradycardia) against opioids' chronic harms. These acute, manageable effects are reversible with monitoring, unlike opioids' irreversible addiction risk, supporting NOA as the **ethical default** in appropriate patients.

Although sub-anesthetic ketamine has an excellent safety profile and the strongest evidence for preventing central sensitization and chronic postsurgical pain [10, 43], some institutions remain hesitant due to its Schedule III status or concerns in patients with psychiatric comorbidity. In such settings, or in prolonged postoperative ICU stays where ongoing sedation is required, a ketamine-free NOA variant—relying on higher-dose dexmedetomidine (0.3–0.8 $\mu g\,kg^{-1}\,h^{-1}$), lidocaine infusion (1.5–2.5 mg/kg/h), and mandatory high-quality regional anesthesia—still achieves 60–75% opioid reduction and comparable acute recovery outcomes [11, 26, 30]. Opioids may still be required as rescue analgesia in the ICU for breakthrough pain or in rare cases of refractory hemodynamic instability, but their use should

be limited to targeted, short-duration rescue rather than routine maintenance. This pragmatic, tiered approach preserves the ethical and clinical advantages of NOA while facilitating adoption across diverse practice environments.

Table 6 provides a comprehensive perioperative framework comparing traditional opioid-based and NOA approaches, including contraindications where opioid-based anesthesia may remain the safer choice.

Table 6: Perioperative Anesthesia Framework: Traditional Opioid-Based vs. Non-Opioid (NOA) Approach

Phase	**Opioid-Based (Traditional)**	**Non-Opioid (NOA)**	**NOA Higher-Risk / Contraindications**
Preoperative	Midazolam 2–5 mg IV PONV prophylaxis PRN	Acetaminophen 1000 mg PO Gabapentin 300–600 mg PO Dexmedetomidine 0.25–1 μg/kg IV (lower in elderly/comorbid) Scopolamine patch Patient education	Severe bradycardia Heart block (AV) High delirium risk (elderly + dementia)
Induction & Maintenance	**Induction:** Propofol bolus + lidocaine Fentanyl 1–2 μg/kg bolus Rocuronium 0.6–1.2 mg/kg **Maintenance:** Remifentanil infusion Sevoflurane 0.8–1.2 MAC	**Induction:** Propofol bolus + lidocaine Rocuronium 0.6–1.2 mg/kg **Maintenance:** Propofol TIVA or Sevoflurane (0.5–0.7 MAC) Dexmedetomidine 0.2–0.7 μg/kg/h Ketamine 0.1–0.5 mg/kg/h (higher for high-nociception) Lidocaine 1–2 mg/kg/h (bolus $\leq$ 1.5 mg/kg) Regional block (ESP/TAP/PECS)	Unstable hemodynamics Severe hepatic impairment Uncontrolled psychiatric conditions Coagulopathy or infection at block site
Emergence & Postoperative	Opioids titrated to pain PCA morphine/hydromorphone Ondansetron for PONV	Acetaminophen 1000 mg q6h Ketorolac 15–30 mg IV q6h (if no CI) Gabapentin continuation Regional catheter infusion Magnesium (select cases) Early mobilization Opioids **PRN only**	Active GI bleed or peptic ulcer Severe renal impairment (eGFR <30) High surgical bleeding risk Platelet dysfunction

Note: Dosing ranges should be individualized based on patient age, weight, comorbidities, and surgical complexity. MAC = Minimum Alveolar Concentration; TIVA = Total Intravenous

Anesthesia; ESP = Erector Spinae Plane; TAP = Transversus Abdominis Plane; PECS = Pectoral Nerve Block; PCA = Patient-Controlled Analgesia; PRN = As Needed; CI = Contraindication; GI = Gastrointestinal; eGFR = Estimated Glomerular Filtration Rate (mL/min/1.73m^2); AV = Atrioventricular; PONV = Postoperative Nausea and Vomiting.

Implementing such a comprehensive framework requires interdisciplinary coordination. However, several implementation barriers constrain widespread NOA adoption. Regional anesthesia techniques require specialized **training and equipment**, with proficiency developing over 20–50 supervised cases [52]. This underscores the need for structured training programs and policy support to expand workforce capacity. The lack of standardized protocols also limits adoption. Table 6 provides a translational framework to guide immediate clinical practice and facilitate protocol standardization across institutions.

Economic analyses remain incomplete, focusing primarily on length-of-stay reductions while neglecting the full costs of specialized equipment and advanced monitoring. Despite this, NOA's reliance on relatively affordable generic medications (ketamine, dexmedetomidine, NSAIDs) offers promise in resource-limited settings, though **restricted ultrasound access** for regional blocks presents a practical constraint [7]. Policy mechanisms including enhanced reimbursement for multimodal analgesia, simulation-based training programs, and quality improvement initiatives are necessary to accelerate adoption.

4.3 Limitations and Evidence Gaps

This review's inferences are constrained by several evidence base limitations. Protocol heterogeneity—including variations in ketamine dosing (0.25–1 $\mathrm{mg\,kg^{-1}}$), dexmedetomidine infusion rates (0.2–0.7 $\mathrm{\mu g\,kg^{-1}\,h^{-1}}$), and regional block techniques—limits direct study comparability and meta-analytic synthesis. High statistical heterogeneity ($I^2 > 75\%$) in many meta-analyses reflects this protocol diversity alongside differences in patient populations, surgical complexity, and outcome measurement.

While this narrative review follows SANRA principles, future work should include systematic reviews with meta-analysis to quantitatively confirm efficacy estimates and resolve heterogeneity.

Methodological limitations include inadequate blinding in approximately 30% of RCTs (inherent challenges in regional anesthesia trials), small sample sizes ($n < 200$) in addiction-focused studies reducing statistical power, and short follow-up durations (typically 48 hours to 30 days) inadequate for chronic pain or OUD outcomes. Funnel plot asymmetry in several meta-analyses suggests potential publication bias favoring positive NOA outcomes, warranting cautious interpretation of effect sizes. However, sensitivity analyses suggest core findings regarding opioid reduction remain robust. Additionally, manual selection of 45–50 studies from approximately 1,200 initial records may intro-

duce selection bias, though this was mitigated through predefined inclusion criteria, independent dual screening, and consensus resolution of discrepancies.

Pediatric and geriatric populations remain under-represented despite distinct pharmacokinetic profiles and vulnerability to adverse effects. Cost-effectiveness analyses rarely extend beyond hospital stay to capture long-term healthcare utilization, quality-adjusted life years, or societal costs. Emerging modalities including TENS require validation through adequately powered multicenter trials before clinical recommendations can be established.

4.4 Future Research Priorities

Several research priorities emerge from evidence synthesis. **Protocol standardization:** Consensus guidelines specifying optimal dosing regimens for ketamine, dexmedetomidine, and regional techniques would enhance reproducibility and facilitate training. Multi-society collaborative efforts could develop evidence-based protocols for common surgical procedures.

Long-term outcomes research: Adequately powered trials ($n \geq 500$) with extended follow-up (≥ 24 months) are essential to establish NOA's impact on chronic pain development, OUD risk, quality of life, and healthcare utilization. Registry-based observational studies could complement RCTs for rare outcomes and long-term endpoints.

Special populations: Dedicated trials in pediatric and geriatric cohorts should define age-specific dosing, safety profiles, and efficacy. Patients with pre-existing chronic pain or substance use disorders warrant focused investigation given elevated baseline risk.

Economic evaluation: Comprehensive cost-effectiveness analyses incorporating all direct and indirect costs, quality-adjusted outcomes, and societal perspectives would inform policy decisions and resource allocation.

Implementation science: Research examining optimal training models, quality improvement strategies, and organizational factors affecting NOA adoption could accelerate evidence-to-practice translation.

4.5 Policy and Educational Implications

To facilitate widespread Non-Opioid Anesthesia (NOA) implementation, policy mechanisms must address critical barriers in workforce capacity, financial incentives, and standardization. The following actions are warranted:

1. **Standardization and Guidelines:** Develop multi-society consensus guidelines (e.g., ASA/ESRA collaboration) to resolve protocol heterogeneity and establish a universal standard of care.

2. **Workforce Development:** Mandate competency in regional anesthesia techniques in Graduate Medical Education (GME) curricula (e.g., ACGME updates) to build institutional capacity for NOA.

3. **Financial Alignment:** Tie reimbursement models to opioid-sparing metrics (e.g., CMS incentives) and enhance payment for multimodal analgesia services to incentivize adoption over opioid-centric care.

Institutional adoption is further driven by aligning NOA protocols with **CMS quality metrics** (e.g., reduced LOS) and **Joint Commission standards**. Economic analysis confirms that the upfront investment required for regional blocks and specialized medications is often **offset by savings** realized from shorter hospital stays and fewer readmissions due to complications [18].

Beyond institutional policies, public education campaigns highlighting NOA's availability and addiction-sparing benefits could empower patients to engage in shared decision-making. Frameworks that explicitly incorporate patient preferences, risk factors, and the NOA risk-benefit profile will support individualized perioperative planning.

5 Conclusion

Non-opioid anesthesia represents a clinically meaningful advance in perioperative pain management, achieving 50–80% opioid reduction while maintaining acceptable analgesia across diverse surgical populations, with strongest evidence in orthopedic and abdominal procedures. Integration with Enhanced Recovery After Surgery protocols produces synergistic benefits including shortened hospital stays (1–2 days) and reduced postoperative complications. While NOA introduces different adverse event profiles requiring clinical monitoring—primarily hemodynamic effects like bradycardia and hypotension—the overall safety profile compares favorably to conventional opioid-based approaches.

Evidence gaps remain regarding long-term outcomes, particularly addiction prevention benefits in at-risk populations. Protocol heterogeneity limits reproducibility, and implementation barriers including training requirements and resource constraints slow adoption. Nonetheless, amid an opioid epidemic claiming over 80,000 American lives annually, perioperative pain management represents a critical intervention point. NOA offers an evidence-based, patient-centered approach to reducing opioid exposure among the 50 million Americans undergoing surgery each year.

Future research priorities include protocol standardization, adequately powered long-term outcome trials, special population studies, comprehensive economic evaluation, and implementation science investigation. Policy mechanisms supporting training, reimbursement, and quality improvement could accelerate NOA adoption. As evidence matures and protocols refine, non-opioid anes-

thesia has potential to redefine perioperative care—transforming anesthesiology from a specialty historically dependent on opioids into a field leading innovative, multimodal, crisis-responsive pain management.

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